

ESCHAROTOMY SKILL GUIDE

BURNS TO TORSO ARE LIKELY ASSOCIATED WITH AIRWAY BURNS. IN THIS INSTANCE, PERFORM DEFINITIVE AIRWAY AND OXYGENATION FIRST THEN TORSO ESCHAROTOMY, THEN LIMB PROCEDURE.

INDICATIONS FOR ESCHAROTOMY

TORSO

CIRCUMFERENTIAL FULL THICKNESS BURNS THAT RESTRICTS CHEST AND/OR ABDOMINAL MOVEMENT WHICH CAUSES VENTILATORY COMPROMISE.

LIMB

CIRCUMFERENTIAL FULL THICKNESS BURN THAT CAUSES CIRCULATORY COMPROMISE DESPITE LIMB ELEVATION. DO NOT USE WHEN LIMB IS DEAD.

PAEDIATRICS PREDOMINANTLY USE ABDOMEN MUSCLES TO BREATHE, SO RESPIRATORY COMPROMISE CAN OCCUR IN CIRCUMFERENTIAL ABDOMINAL BURNS.

ESCHAROTOMY PROCEDURE

PREPARATION

KIT DUMP: CHLORAPREP WIPES, SCALPEL, STERILE GAUZE, CLING FILM

PATIENT: SUPINE WITH 360 ACCESS. CONSIDER SEDATION/ANALGESIA

IDENTIFY & CLEAN LANDMARKS BELOW IN GREEN

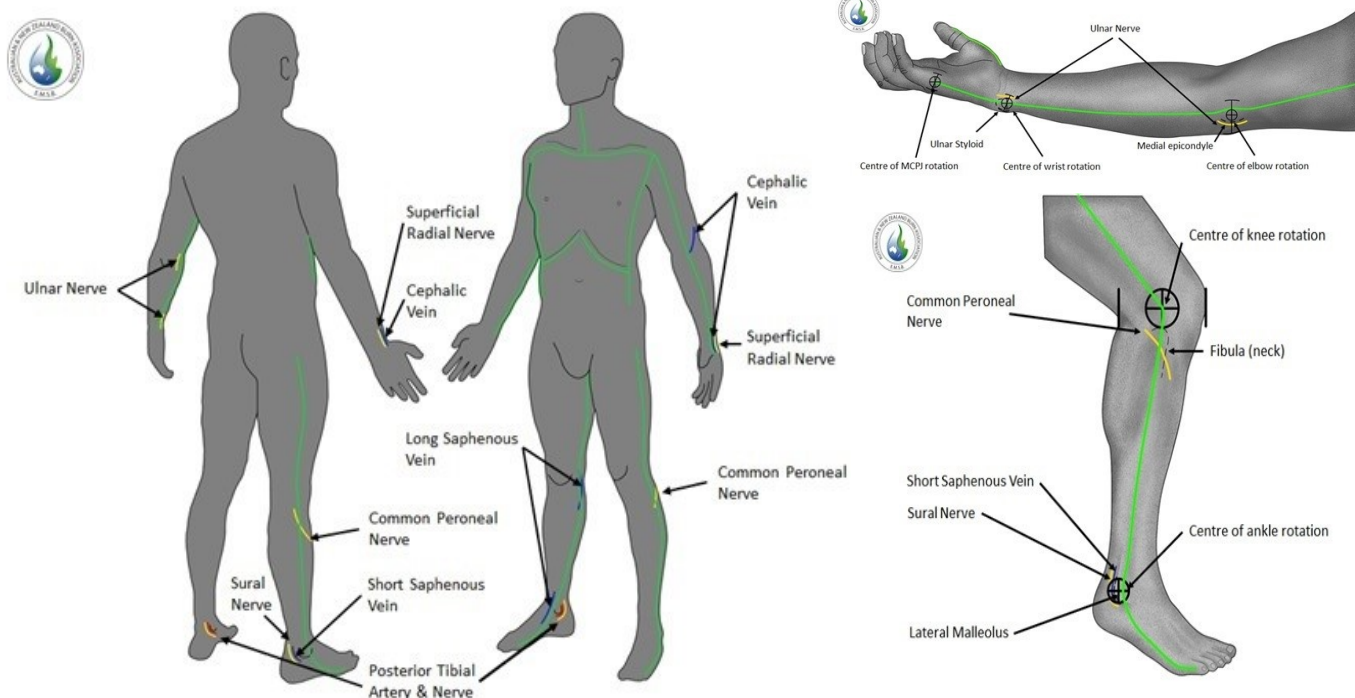
PROCEDURE

MAKE INCISIONS ALONG THE GREEN LINES SHOWN BELOW. INCISIONS SHOULD BE SKIN DEPTH ONLY (SHOULD SEE FATTY TISSUE).

INCISION SHOULD EXTEND ABOVE AND BELOW BURN INTO UNBURNT SKIN (CONSIDER LOCAL ANAESTHETIC IN UNBURNT SKIN)

RUN FINGER ALONG WOUND TO ENSURE NO REMAINING SKIN TIGHTNESS.

PLACE STERILE WET GAUZE INTO INCISIONS AND LAY STRIPS OF CLING FILM OVER WOUNDS TO PROTECT FROM ENVIRONMENT



TORSO ESCHAROTOMY

INCISIONS SHOULD BE ALONG THE MID-AXILLARY LINES EXTENDING DOWN INTO THE ABDOMEN IF REQUIRED.

ONCE COMPLETE, REASSESS AND ENSURE ADEQUATE VENTILATION IS ACHIEVED. BE PREPARED FOR VENTILATORY ARREST. HYPOXIA IS LIKELY THE REVERSIBLE CAUSE.

NECK AND LIMB ESCHAROTOMY

NECK - MIDLINE INCISION USED FOR LIKELY SURGICAL AIRWAY AND TO AVOID VITAL LATERAL NEUROVASCULAR STRUCTURES.

LIMB INCISIONS SHOULD BE DONE MEDIANLY AND LATERALLY AVOIDING AREAS ABOVE. AVOID THIS PROCEDURE IN THE SETTING OF DEAD LIMB (SEE CRUSH INJURY).